

Approach To Dysmenorrhea And Premenstrual Tension

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Objectives :

- Understand the causes, investigation and treatment of dysmenorrhoea.
- Describe the common effects and management of premenstrual syndrome (PMS).

Dysmenorrhoea:

- is defined as painful menstruation. It is experienced by 45–95% of women of reproductive age.
- **Primary dysmenorrhoea** describes painful periods since onset of menarche and is unlikely to be associated with pathology.
- **Secondary dysmenorrhoea** describes painful periods that have developed over time and usually have a secondary cause.

Aetiology of secondary dysmenorrhoea

- Endometriosis: is defined as endometrial tissue lying outside the uterine cavity.
- Adenomyosis: presence of islands of endometrial tissue are found deep within the underlying myometrium
- Pelvic inflammatory disease .
- Cervical stenosis and haematometra (rarely).

To ascertain the actual severity of the pain, the following questions may be useful:

- Do you need to take painkillers for this pain? Which tablets help?
- Have you needed to take any time off work/school due to the pain?
- Some primary dysmenorrhoea is associated with flushing and nausea, which may be prostaglandin related.
- It is important to distinguish between menstrual pain that precedes the period (a vital clue in endometriosis) and pain that only occurs with bleeding.

- Other important clues about the aetiology include pain that occurs with passage of clots, in which case medication to reduce flow may be effective.
- Secondary dysmenorrhoea may be associated with dyspareunia or AUB, which may point towards a pathological diagnosis.
- Certain signs associated with endometriosis include a **pelvic mass** (if an endometrioma is present), a **fixed uterus** (if adhesions are present) and **endometriotic nodules** (palpable in the pouch of Douglas or on the uterosacral ligaments).

- An enlarged uterus may be found with fibroids.
- Abnormal discharge and tenderness may be seen with PID.
- **‘Red flags’** in the expression of dysmenorrhoea lead the clinician to suspect serious pathology and include an **abnormal cervix** on examination, **persistent PCB or IMB**, which may indicate endometrial or cervical pathology, or a pelvic mass that is not obviously the uterus.

Investigations:

- High vaginal and endocervical swabs.
- TVUSS scan endometriosis or adenomyosis.
- Diagnostic laparoscopy: performed to investigate secondary dysmenorrhoea:
 - when the history is suggestive of endometriosis;
 - when swabs and ultrasound scan are normal, yet symptoms persist;
 - when the patient wants a definite diagnosis or wants reassurance that their pelvis is normal.
- If features in the history suggest cervical stenosis, ultrasound-guided hysteroscopy can be used to investigate further (should not be routine).
- Laparoscopy for primary dysmenorrhoea should not usually be performed.

Management:

- Non-steroidal anti-inflammatory drugs (NSAIDs): effective in a large proportion of women. Some examples are naproxen, ibuprofen and mefenamic acid.
- Hormonal contraceptives: COCP is widely used
- Progestogens, either oral (desogestrol) or parenteral (medroxyprogesterone, etonogestrel) may be useful to cause anovulation and amenorrhoea.
- LNG-IUS: esp endometriosis and adenomyosis. It is often used as a first-line treatment before laparoscopy.

- Lifestyle changes: a low fat, vegetarian diet may improve dysmenorrhoea. exercise may improve symptoms by improving blood flow to the pelvis.
- Heat: there is strong evidence to prove its benefit.
- GnRH analogues: this is not a first-line treatment prolonged management due to the resulting hypo-oestrogenic state. if awaiting hysterectomy or as a form of assessment as to the benefits of hysterectomy.
- Surgery: signs or symptoms of pathology such as endometriosis may warrant surgical laparoscopy to perform adhesiolysis or treatment of endometriosis/ drainage of endometriomas.

Premenstrual syndrome

- The occurrence of cyclical somatic, psychological and emotional symptoms that occur in the luteal (premenstrual) phase of the menstrual cycle and resolve by the time menstruation ceases.
- Premenstrual symptoms occur in almost all women of reproductive age.
- In 3–60% symptoms are severe, causing disruption to everyday life, in particular interpersonal relationships.

Aetiology

- The precise aetiology of PMS is unknown, but cyclical ovarian activity and the effects of oestradiol and progesterone on certain neurotransmitters, including serotonin, appear to play a role.

History and examination

The cyclical nature of PMS is the cornerstone of the diagnosis.

A symptom chart, to be filled in by the patient prospectively, may help.

BOX 3.3: Symptoms of PMS

- Mood swings
- Feeling depressed, irritable or bad tempered
- Feeling anxious, upset or emotional
- Tiredness or difficulty sleeping
- Headaches
- Changes in appetite and food cravings
- Feeling clumsy
- Fluid retention and feeling bloated
- Changes to skin or hair
- Sore or tender breasts

Management:

- Simple therapies: include stress reduction, alcohol and caffeine limitation and exercise.
- Medical treatments:
 - 1- COCP: the most effective regime appears to be bicycling or tricycling pill packets (i.e. taking two or three packets in a row without a scheduled break);
 - 2- transdermal oestrogen: this has been shown to significantly reduce PMS symptoms, by overcoming the fluctuations of the normal cycle.

- GnRH analogues
- selective serotonin-reuptake inhibitors (SSRIs).
- Hysterectomy with bilateral salpingo-oophorectomy.
- Vitamins: initial studies suggest that magnesium, calcium and isoflavones and vitamin B6 may be useful in treating PMS.

Alternative therapies:

- initial results of St John's Wort are promising, particularly in improving mood. Although Evening Primrose oil is commonly used, there is no evidence to support this treatment for PMS;
- cognitive-behavioural therapy (CBT): CBT appears to be particularly effective when combined with SSRIs.

Mild to moderate PMS

Encouragement of healthier lifestyle, improved nutrition and regular exercise

Less fat, sugar, salt, caffeine and alcohol
Frequent starchy meals, preferably high in fibre
More fibre, fruit, vegetables

Severe PMS sometimes improves with treatments from the first two levels, but may require more aggressive forms of management sooner

Stress management:

- Relaxation
- Yoga
- Meditation
- Breathing techniques

Counselling/support:

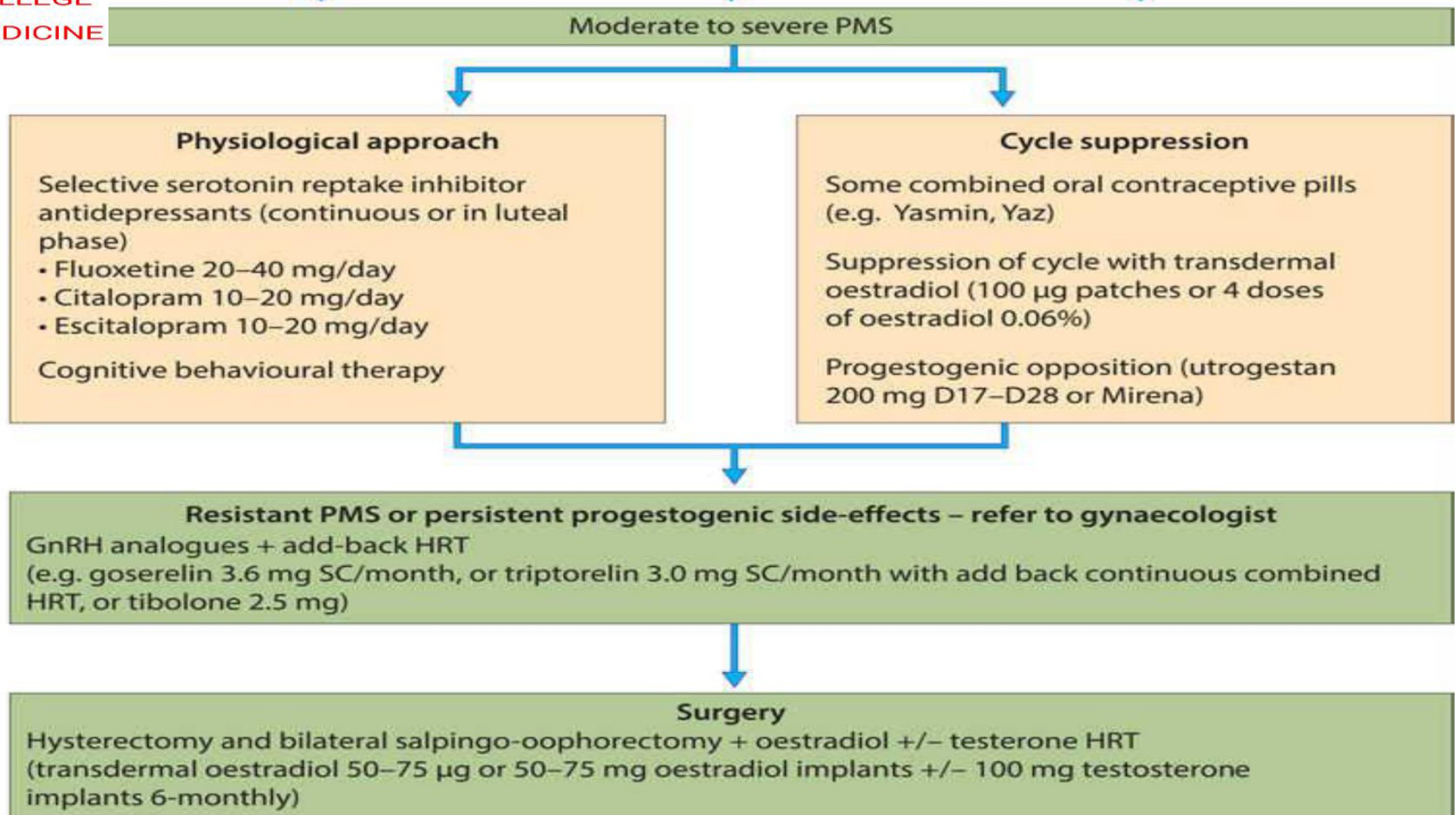
- Family
- Friends
- Professional counsellor
- NAPS

Complementary therapies:

- *Vitex agnus castus*
20–40 mg/day
- Red clover isoflavones
40–80 mg/day
- St John's wort (beware drug interactions)

Vitamin and minerals:

- Vitamin B6, maximum 50 mg/day with GP supervision
- Magnesium 250 mg/day
- Calcium 1 g/day + vitamin D 10 µg/day, especially for migraine



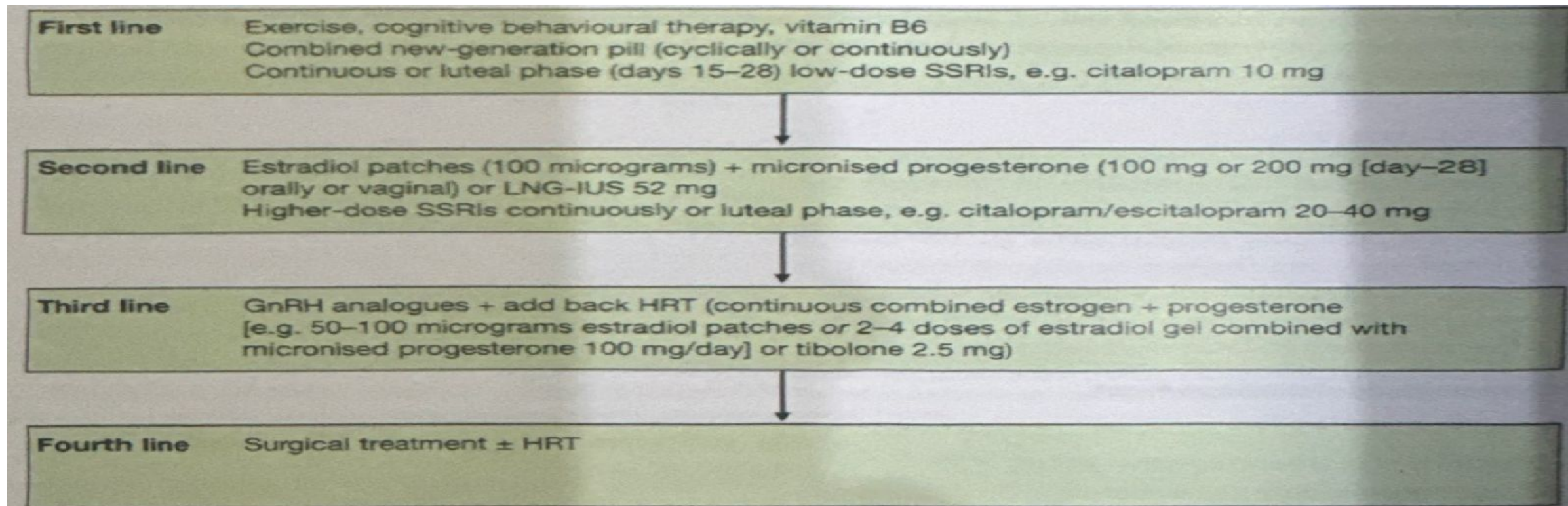


Figure 3.7 Algorithm for the treatment of premenstrual syndrome (PMS). (GnRH, gonadotrophin-releasing hormone; HRT, hormone replacement therapy; SSRI, selective serotonin reuptake inhibitor.) (From Royal College of Obstetricians and Gynaecologists (2017). Green-top guideline No 48: Management of premenstrual syndrome. *BJOG: An International Journal of Obstetrics & Gynaecology*, 24(3): e73–e105. With permission from John Wiley & Sons.)



Thank you







